

Patient ID: PA03	Patient Name: Anonymous
Age: 56 Years	Sex: F
Accession Number:	Modality: MR
Referring Physician:	Study: MRI LUMBAR SPINE +WSS
Study Date:	

MRI LUMBO-SACRAL SPINE WITH WHOLE SPINE SCREENING

Protocol:

Sagittal and Axial T1 and T2 Wt sequences

Coronal STIR sequence on 1.5tesla

H/O BACK PAIN

OBSERVATIONS:

Moderate scoliosis with convexity to left side

Modic type II endplate degenerative changes in L4.5, L5/S1

Otherwise The Vertebral bodies show normal height

No evidence of osteolysis/ osteosclerosis.

The visualised cord is normal in calibre and signal intensity.

Conus is normal in size and signal intensity

No extradural/intradural/ intramedullary mass lesion.

No pre or para vertebral soft tissue mass lesion seen.

Screening SI joints appear normal

Intervertebral discs

Significant changes of lumbar spondylosis seen with multilevel facet arthrosis end plate degenerative changes and peridiscal osteophytes

L4-L5 shows diffuse disc bulge with biparacentral and foraminal herniation and central protrusion compounded with facet arthrosis and ligamentum flavum hypertrophy causing bilateral lateral recess and neural foramen narrowing and compression over thecal and bilateral traversing and exiting nerve roots, bilateral neural foramen narrowing also seen

L5/S1 shows diffuse disc bulge with biparacentral and foraminal herniation compounded with facet arthrosis and ligamentum flavum hypertrophy causing indentation /moderate compression over thecal sac and bilateral traversing nerve Roots , bilateral neural foramen narrowing seen with indentation over exiting nerve roots

.L3/4,L2/3 shows bulge with biparacentral and foraminal herniation and central protrusion compounded with facet arthrosis and ligamentum flavum hypertrophy causing bilateral neural foramen narrowing and indentation over thecal and bilateral traversing nerve roots .bilateral neural foramen narrowing also seen

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L1/2 shows diffuse disc bulge compounded with facet arthrosis and ligamentum flavum hypertrophy causing mild indentation over thecal sac With no neural compromise

ANTEROPOSTERIOR CANNAL DIMENSIONS ARE AS FOLLOWS:

L1/2 15.5MM

L2/3 13.3MM

L3/4 14.5MM

L4/5 10.7MM

L5/S1 10.5MM

Screening cervical spine:

Straightening of cervical spine

Alignment is normal.

The Vertebral bodies are normal in height and signal intensity

No fracture collapse/ sclerosis

Disc osteophyte complexes from C3/4 to C6/7 most predominantly at C5/6 indenting the anterior arachnoid space with bilateral neural foramen narrowing

Otherwise rest of the Intervertebral discs are normal.

Pedicles, laminae and spinous processes are normal.

Otherwise the cervicodorsal cord and vertebra are normal

Screening dorsal spine:

The Vertebral bodies are normal in height and signal intensity

No fracture collapse/ sclerosis

Multilevel disc bulges ligamentum flavum hypertrophy noted in the dorsal spine more prominent at midthoracic levels though with no myelopathy

Otherwise rest of the Intervertebral discs are normal.

Pedicles, laminae and spinous processes are normal.

Otherwise the cervicodorsal cord and vertebra are normal

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Impression:

Moderate scoliosis with convexity to left side

L4-L5 shows diffuse disc bulge with biparacentral and foraminal herniation and central protrusion compounded with facet arthrosis and ligamentum flavum hypertrophy causing bilateral lateral recess and neural foramen narrowing and compression over thecal and bilateral traversing and exiting nerve roots, bilateral neural foramen narrowing also seen

L5/S1 shows diffuse disc bulge with biparacentral and foraminal herniation compounded with facet arthrosis and ligamentum flavum hypertrophy causing indentation /moderate compression over thecal sac and bilateral traversing nerve Roots , bilateral neural foramen narrowing seen with indentation over exiting nerve roots